

September 27, 2016

Division of Dockets Management (HFA-305)
Food and Drug Administration
5630 Fishers Lane, Rm. 1061
Rockville, MD 20852

Re: Docket No. FDA-2015-D-3719 for “Draft Guidances Relating to the Regulation of Human Cells, Tissues, and Cellular and Tissue-Based Products; Rescheduling of Public Hearing; Request for Comments”

Dear Sir or Madam:

My name is Sheila Sablon DeCastro and I am a nurse practitioner at Massachusetts General Hospital and a Consulting Clinical Director to the Tissue Program at Beth Israel Deaconess Hospital-Plymouth. I have over 15 years of experience in clinical, regulatory affairs and tissue donation in regenerative medicine and wound care. I hold a Master of Science Degree in Drug Regulatory and Health Policy from the Massachusetts College of Pharmacy and Health Sciences and a Master of Science in Nursing degree from Boston College. These personal reflections on the proposed draft guidances do not reflect the opinions of Massachusetts General Hospital or Beth Israel Hospital-Plymouth.

The draft guidance documents are urgently needed and a major step forward in providing clarity for the manufacturers of HCT/Ps and healthcare providers to the benefit of patients. Although the regulatory scheme set forth in Part 1271 works well for traditional allograft products, such as cadaver skin used to cover burns, the tiered, risk-based approach laid out in Part 1271 is not functioning as it was intended. The market is saturated with products that are represented as Section 361 HCT/Ps but may not actually meet the criteria of Section 1271.10.

As a threshold matter, the actual status of these products is not ambiguous under existing policy and precedent. These guidances simply compile prior FDA policy interpretations, as discussed in preamble language to proposed and final rules, Tissue Reference Group decisions, and various untitled letters issued over the past several years. Nevertheless, the guidances are needed because certain segments of the allograft industry have disregarded applicable precedents or have chosen to leverage vague language from these sources to provide a rationale for marketing these products without FDA premarket oversight.

This regulatory gap creates a potential safety problem in that it permits the distribution of cell therapy products without FDA oversight. Because these products have not been demonstrated, with valid scientific evidence reviewed by FDA on a premarket basis, to be clinically safe and effective, health care providers are becoming surrogate safety and efficacy reviewers. We are put in a position of determining the safety and efficacy of the products based on white papers and published literature and may be induced to make clinical decisions to the detriment of patients. Patients may receive treatments that do not do what they claim, or may not receive FDA-approved products that have been shown to be safe and effective.

The remainder of my remarks concern the homologous use draft guidance. First, I want to emphasize that the distinction between structural and non-structural tissues is not novel. In the preamble to the section 1271.10 regulations, FDA expressly affirmed the continuing validity of the concept for the application of the term homologous use, notwithstanding the removal of the concept from the definition. The distinction makes clinical sense because it is useful in distinguishing HCT/Ps for which clinical data are necessary from those whose safety and efficacy are readily apparent.

Second, the guidance is not arbitrary or capricious by virtue of providing for the disparate treatment of similar products. In particular, the argument that micronized amniotic tissue must be treated the same as ground bone is premised on a false equivalency, because while they are both structural tissues, micronized amniotic tissue (unlike ground bone) is not intended for a structural purpose. For this reason, there are meaningful differences between the products and disparate treatment is appropriate.

Third, it is appropriate (and consistent with historical precedent) for the guidance to take the position, as it does, that homologous use requires a tissue to be intended for the same basic function in both the donor and the recipient. In particular, the guidance appropriately asserts that tissues that are structural in the donor must be intended to perform a structural function (i.e., nonmetabolic or biochemical) in the recipient. When FDA finalizes this guidance, it should clarify that reducing inflammation or scarring are not homologous uses of tissues which did not perform this function in the donor and that, in particular, reducing scarring and inflammation are not homologous uses of amniotic tissue or cadaver skin.

Finally, I urge FDA, when it finalizes the guidance, to expand the discussion of its existing regulation which provides that “intended use” may be determined by referring not only to advertising and labeling but also to other indications of the manufacturer’s objective intent. It has been longstanding Agency policy that a product’s intended use can be inferred, even in the absence of express claims, when the product’s actual uses are well-known and understood by the product’s end users. FDA should reiterate this principle in the draft guidance, to make clear that the Agency has the legal authority to take action to enforce premarket review requirements for HCT/Ps that have been pervasively promoted for nonhomologous uses even when written labeling and advertising has subsequently been “cleaned up.”

Thank you for the opportunity to submit these comments.

Sincerely,

Sheila S. DeCastro, MS, AGPCNP-BC